

GOVERNMENT TIRUVANNAMALAI MEDICAL COLLEGE

DEPARTMENT OF OBSTETRICS & GYNAECOLOGY

Anemia in Pregnancy

Mrs Tamilselvi, a 20 yrs old coming from keelpennathur, who is home maker, lower socio economic class (obstetric score : primi) with 8 months of amenorrhoea came to OPD c/o pain in the abdomen.

She is booked and immunised.

LMP 12/2/21

EDD 19/11/21

H/OPI

Patient was apparently normal before one week after which she developed diffuse abdominal pain.

Insidious in onset

Intermittent and non progressive in nature

H/O fatigue and lethargy for last one week

H/O headache

No H/O of blurring of vision

No H/O vomiting

No H/O reduced urinary output

No H/O breathlessness, palpitations and chest pain

No H/O pedal edema

No H/O fever with chills and rigor, burning micturition

No H/O passage of worms in stools

No H/O trauma, hemoptysis, hematemesis, melena

No H/O bleeding or leaking oer vagina

No H/O bleeding per rectum

No H/O hematuria, facial puffiness

No H/O yellow discoloration of urine, skin or eyes

No H/O bleeding disorder

No H/O evening rise in temperatures or contact with TB patients

No H/O prior blood transfusion

No H/O bleeding / draining PV

Fetal movements are well perceived

Past history

She is known case of asthmatic (not using inhaler now)

Not a K/C/O DM, HTN, TB, epilepsy, thyroid disorder and cardiovascular disorders

No H/O blood transfusion

No H/O recent surgeries

N/K/c/O =TB, epilepsy, HPT, asthma, thyroid disorder.

Menstrual history

She attained menarchy at the age of 14 yrs.

Regular 28 days cycle, menstrating 3 days, changes 3 to 4 pads per day not associated with clots and not associated with pain.

Marital history

She married at the age of 19 yrs

Non consanguineous

No H/O consumption of OCP.

Obstetrics history

Primi gravida

Pregnancy was confirmed by UPT at 6 weeks of amenorrhoea

Booked and immunised

First trimester

H/O spontaneous conception

Pregnancy detected at home with urine pregnancy test after one month of her missed periods.

First dose of Td given

Dating scan done at 12th week

NT scan done

H/O breathelessness

No H/O fever with rashes

No H/O hyperemesis gravidarum

No H/O radiation exposure

No H/O drug intake

No H/O pain abdomen

No H/O bleeding PV, burning micturition

Second trimester

Quickening felt at 6th week

Second dose of Td given

Anamoly scan at 20th week

H/O easy Fatigubility

H/O iron and folic acid supplementation

H/O whitish discharge not associated with foul smelling

H/O burning micturition

No H/O headache, blurring of vision, vomiting, epigastric pain, decreased urine output

No H/O increased appetite, thirst, increased frequency of micturition

Third trimester

Continue to perceive fetal mmovements

No H/O pain abdomen

No H/O bleeding PV, draining PV

No H/O blood transfusion

Growth scan done

No H/O suggestive of diabetes, hypertension

Personal history

She consumed mixed diet

Normal bowel and bladder habits

Normal sleep pattern

No H/O drug allergy

Family history

Not a K/C/O DM, HTN, TB, epilepsy, thyroid disorders, cardiovascular disorders

No H/O previous surgeries

No H/O trauma

No H/O repeated blood transfusion

General examination

The patient was comfortable and cooperative, oriented, moderately build, moderately nourished

Patient was anemic

No icterus

No cyanosis

No clubbing

No generalised Lymphadenopathy

Bilateral pedal edema confined to the ankle.

Vitals

PR =85 beats /min

BP=130/70 mm Hg

Temperature=

Weight =

Height=

BMI=

No angular stomatitis, glossitis, cheilosis

Nails: no platonychia, no koilonychia

Thyroid appears to be normal

Spine appear to be normal

Breast appears to be normal.

Abdominal examination

After getting concerned from the patient

After emptying bladder at a supine position, thighs are slightly flexed at hip,

Inspection

Longitudinally distended

Umblicus in midline everted

All quadrants moves equally with respiration

Flank full, hernial orifice free

No scar, cyanosis, dilated veins, visible pulsations.

Palpation

Legs semiflexed correcting the dextro rotation

SFH :

Abdominal girth

Fundal height

After emptying bladder at supine position, legs extended and correcting dextra rotation

Fundal height corresponds to 32 weeks of gestation

Fundal grip

Broad soft, non independently fetal part is present. Suggestion of breech

Umbilical grip

Left – multiple small nodular ballatable felt limb buds

Right -smooth curved uniform resistancefelt suggestive of back or spine

First pelvic grip

Hard, round, independently ballatable fetal part felt at lower pole of uterus mobile propable of head

Second pelvic grip

First Pelvic grip is confirmed

Head is mobile.

Auscultation

Fetal heart sound heard

Sounds are heard at right spino umbilical line below the umblicus

140 beats /min, regular in rhythm

Other systemic examination -N

CVS : s1, s2 heard, no murmur,

RS: normal vesicular breath sounds heard. No added sounds

CNS: no focal neurological deficit

Summary

A 20 yrs old primi booked and immunised belonging to low socio economic status with LMP 12/2/21 and EDD 19/11/21 c/O of lower abdominal pain with fatigue and headache for the past one week.

O/E patient was anemic with single live fetus longitudinal lie. Cephalic presentation with good fetal heart sound not in labour came for institutional evaluation and treatment.

Diagnosis

A 20yrs old primi with 32weeks of gestation with single fetus live longitudinally, cephalic presentation with good fetal heart sound as diagnosed a case of moderate anemia admitted for 3weeks not in labour with no other medical and obstetrics complication came for institutional evaluation and treatment.

Investigation

Blood : CBC, blood grouping and typing

 HB-7% bleeding time and clotting time

Blood : sugar, urea, creatinine

Urine :albumin

 Sugar

 Pus cells

Urine culture and sensitivity

Peripheral smear

Stool examination

USG

HIV

HBSAG.

VDRL

PREECLAMPSIA

32 yrs old Mrs. Mahalakshmi w/o chellam coming from Tharadapat from TVM, G2P1D1, belongs to lower middle class, with 7 months of amenorrhea, booked and immunised for Td vaccine, with regular cycle, LMP(13/03/2021) ,EDD(20/12/2021) came to OPD with C/O dizziness and headache for 1month, swelling of leg for 1week

H/O presenting illness:

The patient was apparently normal before 1 month after which she developed dizziness and headache and swelling of feet for past 1 week

Which is insidious in onset ,does not relieved by rest

No H/O diminished urine output

No H/O diminished fetal movement

H/O headache

No H/O visual defect, giddiness, epigastric pain, vomiting

No H/O pain abdomen

No H/O bleeding pervagina

No H/O draining pervagina

Past history:

Not a known case of DM, HT, TB, Asthma, Epilepsy, RHD

H/O thyroid disorder for past 1 month

No H/O previous surgeries, bleeding disorder

No H/O blood transfusion

Menstrual H/O:

She attained menarche at a age of 15 yrs

Irregular cycles , menstruating 5 days changes 4-5 per day

Not associated with clots

Associated with pain

Marital H/O:

Married since 11 yrs

Married at 24 yrs ,non- consanguine

Conceived after 10 years of marriage

Obstetric H/O: G2P1D1

I st trimester :

H/O spontaneous conception
Pregnancy confirmed by urine pregnancy at 40 days
1st dose of Td was given
H/O nausea and vomiting
No H/O exanthematous fever
No H/O pain abdomen
No H/O pervaginal bleeding
No H/O draining pervagina
No H/O drug intake, radiation exposure
Dating scan done at 12 weeks
NT scan done

II nd trimester:

Quickening felt at 5 th month
USG done at 5 th month -anomaly scan
2nd dose of Td was given
No H/O fatigability, breathlessness and palpitation
H/O of IFA tablet
No H/o of blurring of vision, headache, abdominal pain, leg swelling
No H/o of polyuria, polydypsia, polyphagia
No H/O abdominal pain
No H/o bleeding per vagina
No H/O draining pervagina

III rd Trimester:

Able to perceive foetal movement
H/o of leg swelling,
H/o of headache,
No H/O pain abdomen
No H/o of bleeding per vagina, draining pervagina
USG done at 7th month

Patient was diagnosed to have hypertension,before 2 days and he is on treatment(labetalol 100mg TDS)

Family H/O:

No H/o of DM,HT,thyroid disorder, epilepsy,asthma in family

No H/O similar disease in family

Personal H/o:

Consumes mixed diet,

Normal bowel and bladder habits

Normal sleep pattern

General Examination:

Patient conscious, comfortable, co-operative, oriented, moderately built and nourished

Not anaemic

No icterus

No cyanosis

No clubbing

No lymphadenopathy

Bilateral pitting pedal oedema upto knee level

Breast,thyroid,spine -normal

Gait normal

Weight:pre-pregnancy(61kg)

Pregnancy (75kg)

Height:145cm

BMI=38(obese)

Vitals:

Temperature:37.4°C

Pulse rate:62 per minute

Blood pressure:148/98 mm Hg

Spo2:98%

CVS:S1,S2- normal ,no murmur

RS:normal vesicular breath sound heard,no added sounds

CNS: no focal neurological deficit

Abdominal examination:

After verbal consent , after emptying bladder ,patient lie in supine position with semiflexed leg exposed from xiphisternum to pubic symphysis

Inspection:

Abdomen longitudinally distended

Flanks full

Umblicus flushed to skin and everted

Linea nigra seen

Stria gravidarum seen

No scar ,no sinus , no dilated veins

Palpation:

Legs extended and correcting dextro rotation , empty bladder , fundal height corresponds to 38 weeks of gestation

SFH:

Abdominal girth:

Fundal grip: Thighs are slightly fixed at hip , broad ,soft ,non independently balloatable fetal part is present -S/O breech

Umbilical grip: Right side – smooth curved , uniform resistance felt,probably spine

Left side- multiple small nodules felt, probably limb buds

First pelvic grip:

Hard ,round, independently balloatable fetal part felt at lower pole of uterus probably head

Second pelvic group: Head is mobile

Auscultation:

Fetal heart sound is felt at left spinoumbilical line

Rate :120 -140/min

Regular in rhythm

Summary:

A 32 yr old Primigravida belonging to lower middle class , booked and immunised with LMP(13/01/2021),EDD(20/12/2021) with 38 weeks of gestation with consultation of swelling of feet

O/E bilateral pitting pedal oedema present

Fundal height corresponds to 32 weeks of gestation ,single live fetus , longitudinal lie with vertex presentation , good fetal heart sound came for institutional evaluation and treatment

Diagnosis:

A 32 yrs old Primigravida with 28 weeks of gestation has single fetus lying longitudinally ,cephalic presentation with good fetal heart sound is diagnosed as a case of **Pre eclampsia** with no other medical and obstetrics complication came for institutional evaluation and treatment

Investigation:

Hemoglobin ,PCV ,

Blood urea, sugar ,serum creatinine

Urine – albumin ,sugar

Urine culture and sensitivity

Blood grouping and typing

BT, CT, OGTT,thyroid function test

HIV,VDRL,HbsAg

Peripheral smear

Specific Renal function test

Liver function test

Fundus examination

USG abdomen

GESTATIONAL DIABETES MELLITUS

Name: devi

Age: 33

Occupation: house wife

Educational status: tenth grade

Husband name: gubendran

Age:43

Occupation: theatre owner

Educational status: 5 th grade

According to modified kuppusamy scale socio- economic status is lower middle class

Obstetric score: G3P2L2

LMP: 10/01/2021

EDD: 17/10/2021

Booked and immunised

Gestational age: 270/7 days= 38 weeks 4 days

Blood group: AB+ ve.

Chief complaints:

Patient was diagnosed as a case of gestational diabetes Mellitus at kalasapakkam PHC and referred for institutional evaluations and delivery.

H/o presenting illness:

H/o increased blood sugar levels detected by OGTT at 28 weeks of gestation

H/o increased thirst

H/o frequent micturition

No H/o increased food intake

No H/O of UTI at 3 rd month which lasted for 2 months

No H/O abdominal pain

No H/o bleeding pv

No H/O draining pv

H/o vaginal discharge

No H/O Skin infection

Past history:

She is known case of asthmatic

Not a k/c/O, DM, HTN, TB, epilepsy and thyroid disorders

No H/O blood transfusion

No H/O recent surgeries

No H/O infections / malignancy in the past

Menstrual H/O

She attained menarche at 14 years regular cycles of 22-25 days with 3-5 days flow and changes 2-3 pads per day

Not associated with dysmenorrhea

No H/O passage of clots

Marital H/O

Married since 17 years Consanguinous marriage

3rd degree

Obstetric H/o (G3P2L2)

1st pregnancy

4 years after marriage delivered by normal vaginal delivery, delivered at CMC vellore

No H/O GDM

Female child of birth weight -2.5 kg

Cried immediately after birth

Breast feeding within 4 hrs and continued for 1 year

2nd pregnancy:

6 yrs after marriage delivered by normal vaginal delivery

Delivered at GTVMCH

No H/O GDM

Female child of birth weight -2.7 kg

Cried immediately after birth

Breast feeding within 5 hrs and continued for 1 year

H/O present pregnancy:

First trimester:

Spontaneous conception diagnosed by urine pregnancy test by herself at 10 th day after missed periods.

Booked & immunised

Confirmed by dating scan done at 9 th week

Folic acid tablets were taken

H/o burning micturition

H/o increased volume of micturition

No H/ o excessive vomiting

No H/o bleeding pv& draining pv

No H/o abdominal pain

No H/o fever with rashes.

No H/o drug intake

No H/o radiation exposure

Second trimester

Quickening felt at 5th month

2 doses of tetanus toxoid taken at 3rd and 4th month

Iron and folic acid and calcium tablets taken regularly

Anamoly scan was done at 5th month and report was normal

H/O increased thirst,increased frequency of micturition

No H/O headache, blurring vision, epigastric pain, vomiting, pedal edema

No H/ O easy fatigability, breathlessness

No H/O abdominal pain

No H/O bleeding pv & draining pv

Third trimester

Continued to perceive fetal movements well

Growth scan was done at 7th month and report was normal

Iron and folic acid were taken

No H/O headache, BOV, epigastric pain, vomiting,Decreased urine output

No H/o abdominal pain

No H/O bleeding pv and draining pv

H/O increased thirst, increased frequency of micturition

Patient came monthly once upto 7 months for AN check up

Then after that 2 weeks once

Contraceptive H/O

No H/O of contraceptive use

Family history:

Her father is a known diabetic

NoH/ O children with congenital malformation

No H/O twinning in the family

No H/O HTN, TB, asthma, in other family members

Personal history:

Patient consumes mixed diet

Normal appetite

Normal bowel habits

Normal sleep pattern

General examination:

Patient is comfortable , cooperative, moderately build and moderately nourished

Patient is not anaemic, no icterus, no cyanosis, no clubbing, no lymphadenopathy

No Bilateral pitting pedal edema present

Breast examination, thyroid examination, spine examination are normal

Normal gait

Height: 165cm

Weight:

Pre – pregnancy _ 79 kg

Present weight_ 86 kg

BMI- 29.01

Other system examination:

CVS: S1 and S2 heard, No murmurs

RS:

Normal vesicular breath sounds heard

No added sounds

CNS:

No focal neurological deficit

Abdominal examination:

After emptying the bladder and after getting consent exposed from xyphi sternum to pubic symphysis

Inspection:

Abdomen uniformly distended- flanks full

Corresponding quadrants move equally with respiration- linea nigra, stria gravidarum present

Umbilicus is midline and everted

No visible scars, sinuses, dilated vein is seen

Palpation:

After correcting dextro rotation

No local rise in temperature

No tenderness

Fundal height corresponds to 38 weeks of gestation and flanks full

SFH:

Abdominal girth:

Leopold's manoeuvre:

Fundal grip: broad, soft, non independently ballotable. Fetal part is felt suggesting breech

Umbilical grip:

On the left smooth curved uniform resistance felt suggesting spine

On the right multiple small nodules felt suggesting limb buds

First pelvic grip:

Hard round independently ballotable mass felt suggesting head at the lower pole uterus

Second pelvic grip:

Converging of hands – so head not engaged

Auscultation:

Fetal heart sounds heard in the left spino umbilical line which is 142 beats / minute

Rhythm – regular

Summary:

A 33 yr old G3P2 L2 belonging to lower middle class, booked and immunised with LMP- 10/01/2021 and EDD- 17/10/2021 referred from PHC with raised sugar levels, increased thirst, frequent micturition. On examination, a single live fetus, longitudinal lie, cephalic presentation with good fetal heart sounds with no other medical and other complications came for institutional evaluation and treatment

Diagnosis:

33yr old gravida 3 with 2 previous term normal vaginal delivery now with 38 weeks 4 days gestation with single live fetus in cephalic presentation, longitudinal lie, good FHS with gestational diabetes Mellitus . Good glycemic control complicating pregnancy with no other medical and obstetric complications for institutional evaluation and delivery

Investigation:

Routine:

Hb, pcv

Blood grouping, typing

Bleeding time, clotting time

Blood: sugar, urea, creatinine

VDRL

HIV

HbsAg

GTT -screening test

Serum TSH

Pap smear

RFT

Specific :

Urine culture sensitivity

Blood sugar monitoring

Urine -ketone bodies

Thyroid profile

HbA1C

USG

Fundoscopy

Management:

Aim:

To maintain 2hrs PPPG in the range of 110-120mg /dl

Meal plan (medical nutrition therapy)

Splitting the usual breakfast into 2 equal halves and consuming the portion with a two hour gap in between

Principles:

Avoid sugar, sweets, tubers, fruit juices

Avoid fasting

Eat to satisfy appetite

Eat more green leaf vegetables

PPPG is less than 120mg /dl under control by meal plan

Continue the meal, repeat 2hrs PPPG once every 4 weeks

Principles of management:

Control of DM

Obstetric management

Timing of delivery

Specialized care

Insulin schedule:

Blood sugar if not controlled initiate insulin therapy

If 2hrs PPPG is greater than 120mg/dl Advice intermediate acting insulin eg. Insulatard or premixed insulin 30/70 four units, 30mins before breakfast

Repeat 2hrs PPPG after 2 weeks, within 120mg/dl continue the same dose

If values are high increase the dose by 2-4 units i.e. 6-8 units, 30 mins before breakfast

Repeat the test every 15 days, titrate the dose to achieve the 2hrs PPPG between 110-120mg/dl

BREECH PRESENTATION

A 21 year old Mrs. Malathi (w/o Mr. Karthikayen) housewife from Thiruvannamalai, belonging to lower socio-economic status G2P1L1, booked and immunised against Td,

regular cycles, LMP 1/1/2021 EDD 8/10/2021 with 9 months of amenorrhoea came for institutional treatment and delivery.

H/o of Presenting Illness.

Perception of fetal movements

No h/o bleeding pv draining pv

No h/o abdominal pain

No h/o headache, blurring of vision, epigastric pain, vomiting and decreased urine output

Menstrual History.

Attained menarche at 14 years of age

Regular cycles 5/28 day, normal flow changes 3-4 pads per day, not associated with pain /clots.

Marital History.

Married since 4 years

Non-consanguineous marriage

Obstetric History.

G2P1L1

1st Trimester.

H/o spontaneous conception

Pregnancy was confirmed by UPT 4weeks.

Folic acid tablets were taken

No H/o pain abdomen

H/o nausea, vomiting

No H/o exanthematous fever with rashes

No H/o drug intake

No H/o radiation exposure

No H/o pv bleeding

No H/o burning micturition

Dating scan done, NT scan done

2nd Trimester.

Quickening felt at 4th month

Anomaly scan done and was normal

Iron, Folic acid, and calcium tablets were taken

No H/o fever with rashes
No H/o radiation exposure or drug intake
No H/o increase thirst, increase frequency of micturition, increase appetite
No H/o headache, blurring of vision, epigastric pain, reduced urine output and vomiting
No H/o easy fatigability, palpitation and breathlessness
No H/o polyuria /polydypsia
No H/o pv bleeding
No H/o increase BP and other Ix are normal
No H/o burning micturition
USG done at 5th month

3rd Trimester.

Perception of fetal movements
No H/o pv bleeding
Iron and calcium tablets taken
Growth scan done
Weight gain in pregnancy
USG done at 7th month

Past obstetric History.

Conceived 2 months after marriage with H/o malpresentation, delivered through LSCS at GTVMMCH, female baby 2.5 kg at birth, cried immediately, no postpartum complication and exclusively breastfeed for 6 months.

Past History.

No H/o Diabetes mellitus, Hypertension, Tuberculosis, Epilepsy, Asthma.
No H/o blood transfusions
No H/o surgery in past
No H/o trauma

Personal History.

Consumes mixed diet
Bowel and bladder habits normal
Normal sleep pattern

Family history

No H/o Diabetes, HTN, TB, Epilepsy, Asthma, rheumatoid heart disease

No relevant family history
No H/o congenital anomalies
No H/o twins in the family

General Examination

Patient was conscious, oriented to time and place, comfortable and cooperative, moderately built and nourished

No anemia, icterus, clubbing, cyanosis, lymphadenopathy

H/o bilateral mild pitting pedal edema

Breast, thyroid, spine examination normal

Gait : zordotic gait

Height :162 cms

Weight:60 kg

BMI : 20.9

Vitals

BP:110/70

PP:88/min

RR:16/min

Temperature :98.4°F

Other system

Cvs:s1 s2 heard, no added murmurs

RS: normal vesicular breath sound, no added sound

CNS:no focal neurological deficit

Obstetric Examination:After getting consent from the patient,emptying the bladder, exposing from xiphisternum to public symphysis.

Inspection:

Patient on supine consent(verbal consent)

Abdomen longitudinally distended, flanks full

Umbilicus flushed to the surface

Stria gravid arum, line a niagra seen

A Suprapubic transverse scar 15 cm healthy scar

No presence of sinus, dilated veins

Palpation: (after emptying the bladder)

1. Fundal height

Fundal height corresponds to 39 weeks of gestation, flanks full

Symphysiofundal height 36 cm

2. Fundal grip:

Hard, round, independently ballotable fetal part – probably head

3. Umbilical grip

Smooth, curved, hard, uniform resistance denotes back on left side

4. I) 1st pelvic grip:

Reveal broad, soft, non independently ballotable fetal part
indication breech in lower pole

II) 2nd pelvic grip:

Breech is not engaged

Auscultation :

Fetal heart sound is heard above the umbilicus on the left side

Summary

A 21 year old G2P1L1, booked and immunised with 9 months of amenorrhoea corresponding to 39 weeks of gestational age with previous Obstetric History of baby delivered through LSCS due to malpresentation. On examination, a single like fetus, longitudinal lie, Breech presentation, fetal heard sounds are well perceived with no other complaints without medial and obstetric complications and not in labour.

Diagnosis

A 21 year old second gravida para one live one who has completed 39 weeks of gestation with previous LSCS due to breech presentation with single live fetus, with good fetal heart rate who has been admitted for institutional deliver Management

Confirm the diagnosis by USG

Delivery plan:

Elective LSCS $\geq$ 38 weeks at GTVMMCH along with sterilisation

Baseline investigation :

Hb, PCV, urine albumin, blood grouping, typing, BT, CT, blood sugar

RFT: blood urea, creatinine

LFT

TFT

HIV, VDRL, HbsAg, HCV

Specific USG

TWIN

Rameeja Bhanu, 35 year old, G4P2L2A1, housewife from Thandarampit, W/O Ahmed Basha, belonging to Lower Middle class, with 8 months of amenorrhoea, LMP-05/03/2021 and

EDD-12/12/2021, B&I came to the OPD with the chief complaints of Lower abdominal pain for the past 2 days.

HPI:

The patient was apparently normal before 2 days after which she developed lower abdominal pain which was sudden in onset, non-progressive, non-radiating, relieved by rest.

No history of bleeding PV.

No history of draining PV.

Able to perceive fetal movements well.

No history of imminent signs suggestive of preeclampsia like headache, blurring of vision.

History of fatigability, breathlessness.

MENSTRUAL HISTORY:

Attained menarche at 14 years

Regular cycles;5/28 day cycle

Changes 3 pads per day.N/A with clots.

MARITAL HISTORY:

Married at 20 years;since 13 years.

Non-Consanguinious marriage.

No history of contraceptive used.

OBSTETRIC HISTORY:

Present Obstetric History: LCB-11 years back

I st Trimester: Confirmed pregnancy by UPT

Spontaneous Conception

Booked and Immunized.

History of nausea and excessive vomiting

History of consumption of Folic acid supplements

Dating scan done

No history of fever with rashes

No history of burning micturition

No history of radiation exposure

No history of bleeding PV, draining PV, Lower abdominal pain

No history of any other drug intake.

Routine investigations done and found to be normal.

2nd Trimester:

Quickening felt at 4 months.

Anomaly scan done

IFA tablets taken

No history of polyphagia, polyuria, polydipsia

No history of headache, blurring of vision, nausea and epigastric pain

History of easy fatigability. Hb concentration was found to be low and parenteral iron given.

No history of bleeding PV, draining PV, pain abdomen

3rd Trimester:

Continued to perceive fetal movements

History of lower abdominal pain

History of bleeding PV

No history suggestive of GDM, PIH

Growth scan taken.

Hb estimation done and found to be anemic.

Past Obstetric History:

1st Pregnancy:

Conceived after 3 years of marriage. Underwent treatment for infertility. Spontaneous full term NVD at Institution.

Outcome: Boy weighed 3 Kg 100gm

Cried immediately after birth. Breastfed after 2 hrs

Immunized

No NICU admission

PP period uneventful.

The boy is 10 years old.

2nd Pregnancy:

Conceived 2 years after 1st child

Spontaneous conception

Spontaneous full term NVD at Institution

Outcome: Girl weighed 3 Kg; Cried immediately after birth; Breastfed within half an hour; PP period uneventful

No NICU Admission

The girl is 8 years old.

3rd Pregnancy:

Conceived 8 years back

History of spontaneous isolate miscarriage at 2nd month

No history of trauma

History of hospitalization and evacuation.

Past History:

No history of previous surgery

No history of trauma

No history of blood transfusions

N/C/K/O DM, Asthma, Tb, CAD

Family History:

No history of DM, HTN, bleeding disease in families

No history of twins in the family

No H/o congenital anomalies

No H/o bleeding disorder

Personal History:

Consumes mixed diet

Normal bowel and bladder

Normal sleep pattern

Examination:

Patient is cooperative, comfortable, moderately built and well nourished.

General Examination:

Patient is anemic

Not icteric

No cyanosis

No clubbing

No pitting edema

Gait, Thyroid, Spine, Breast normal

Arthropometry:

Weight-83kg

Height-150cm

BMI-36.8

Other systems Examination:

Local Examination:

Inspection:

Barrel shaped, over distended abdomen.

Umbilicus in midline, flushed with skin.

Flanks full

Linea nigra seen

Stria albicans gravidorum seen

No scars, sinuses and dilated veins seen

Palpation: Obtaining consent

After emptying bladder with adequate privacy, after correcting dextrorotation,

Fundal height corresponds to 36 weeks of gestation

Fundal grip –with legs semiflexed

Multiple fetal parts felt.

3 fetal poles felt

Hard round independently ballotable mass felt at right side suggestive of fetal head

Umbilical grip- Hard round independently ballotable mass felt probably head.

1st pelvic grip- Broad, round, soft, not independently ballotable

2nd pelvic grip- Hands are diverging- unengaged

Auscultation:

2 distinct fetal heart sound heard; 10 cm apart.

Right-Spinoumbilical line

Left-Side at the level of umbilicus

Summary:

Elderly Gravida(35 Years), G4P2L2A1, regular cycles with 2 previous NVD at term and one spontaneous isolated incomplete abortion, 8 months of amenorrhoea and came with complaints of lower abdominal pain for past 2 days. On examination, pallor present, overdistended abdomen, fundal height 36 weeks of gestation, multiple fetal parts felt, 2 fetus 1st twin in breech presentation with anemia complicating pregnancy with no other medical and obstetric complications came for Institutional evaluation and treatment

Dx: Elderly Gravida G4P2L2A1 regular cycles with previous NVD at term and 1 spontaneous isolated abortion now with 36 weeks of gestation 2 live fetuses longitudinal lie; One breech and one transverse presentation good FHR with multiple pregnancy complication with polyhydramnios. No other medical or obstetric comorbidities came for institutional evaluation.

Ix:

Basic:

1. Hb concentration; blood grouping and typing
2. Urine- albumin and sugar deposits
3. VDRL, HBSAg, HIV screen
4. OGTT
5. TFT
6. BT, CT

Specific:

1.USG

2.NT

[PREVIOUS LSCS](#)

Mrs. parameshwari 25 years old , housewife from thiruvannamalai came to OPD with her husband balakrishna, she belongs to upper middle class of social economic status.

G2P1L1

Booked and immunized

LMP- 13/01/2021

EDD- 20/10/2021

Chief compliants:

H/O of 9 months of amenorrhoea presented with 39 weeks of gestation, a case of post c-section referred from PHC for institutional evaluation and management.

HOPi:

NO History of pain abdomen

No H/O bleeding PV

No H/O draining PV

Able to perceive fetal movements.

No imminent symptoms suggestive of GHT like headache, blurring of vision, epigastric pain, vomiting, decreased urine output.

Past history:

Not a k/c/o – DM, TB, Asthma, HTN, epilepsy

No H/O blood transfusion

No H/O recent surgery

No H/o trauma

Menstrual history:

Age of attainment of menarche -14 yrs

Regular cycle of 20-30 days

Normal flow for 4-5 days

Changes 2-3 pads / day

Not associated with pain or passage of clots

History of copper T insertion following previous pregnancy

Obstetric history:

Past obstetric history:

Same all detailed history to be presented

H/O of present pregnancy:

Booked and immunized G2P1L1- 1st pregnancy.

LMP- 13/01/2021

EDD- 20/10/2021

1st TM

H/O of spontaneous conception

Pregnancy was confirmed after 2 months of missed cycle by urine pregnancy test.

Dating scan was done and found to be normal and corresponding with dates.

Folic acid tablet were taken

No H/O pain abdomen

No H/O bleeding PV

No H/O draining PV

No H/O of fever and rashes / vomiting

No H/O of burning micturition

No H/O drug intake / radiation exposure

Regular AN check up.was done and no abnormalities found in 1st TM.

2nd TM

Quickening felt at 5th month

Anomaly scan was done and found to be normal

2nd dose of TT was taken

Iron, folic acid and calicum are taken

No H/O pain abdomen

No H/O bleeding PV

No H/O draining PV

No H/O headache, blurring of vision, epigastric pain,decreased urinary out put and vomiting

No H/O of increased appetite or thirst

No H/O of easy fatiguability

Regular AN check up.was done and no abnormalities found in 2nd TM.

3rd TM

Continued to perceive fetal movements

Iron, folic acid tablet were taken

Growth scan was done and found to be normal

No H/O pain abdomen

No H/O bleeding PV

No H/O draining PV

No H/O headache, blurring of vision, epigastric pain, decreased urinary output and vomiting

No H/O of increased appetite or thirst

No H/O of easy fatigability

Past obstetrics history

Antenatal events- Booked and immunized at govt hospital

Perinatal events- post term baby, institutional delivery by elective LSCS 3 YRS back on 16/6/18 at 41 weeks of gestation

It was done 7 days after EDD

Indication-prolonged pregnancy with failure of induction

Outcome- alive boy baby ,of 3.1 kgs birth/ wts and well immunised.

No H/o NICU admission

Currently healthy and cried immediately after birth and doing well

No fever/ wound discharge in postoperative period

No blood transfusion

Sutures were removed on 11th day

Postnatal events-uneventful

Personal history- consumes mixed diet

Normal sleep pattern/ sleep cycles

No bowel and bladder movement

No H/O of any addiction

EXAMINATION

GENERAL

Patient comfortable and cooperative , moderately build and afebrile

Not anemic, not icterus, no clubbing, no cyanosis, bilateral pitting pedal edema, no lymphadenopathy.

Breast, thyroid, spine-normal

Gait- lordotic gait

Vitals

Pulse rate-88 per min

BP- 110/80 mmHg

Respiratory rate-23/min

Height -160cm

Wt-pre pregnant-52kgs Pregnant – 63 kgs

BMI-24.6(NORMAL)

SYSTEMIC

CVS – S1 and S2 heard. no murmur heard

RS- Normal vesicular breath sound/ no added sound

Abdominal

After explaining the procedure and getting consent, following examination to be done after emptying the bladder and exposing from xiphisternum to mid thigh.

Inspection

Abdomen longitudinally distended flanks not full

Umbilicus is centered and flush to the skin.

Linea nigra and stria gravidum present.

Previous LSCS scar present.

Horizontal scar of 10cm noted 3cm above the pubic symphysis.

Healed by primary intention, without hypertrophy and keloid formation

No sinus / dilated veins / scar

Hernial orifice is intact

Palpation

No local rise in temperature and tenderness

Fundal height corresponds to the gestational age 32 Weeks with flank -full

SFH:

Abdominal girth:

Fundal grip

Broad,soft ,bulky non independently ballotable – s/o breech.

Umbilical grip

Right lateral grip- smooth curved .hard uniform resistance

Left lateral grip-multiple small nodules felt s/o

First pelvic grip

Hard found independently ballotable mass- s/o fetal head

Second pelvic grip

Finding of first pelvic grip is confirmed

Fingers convergers-head not engaged

No scar tenderness

Auscultation

FHS heard along it spino umbilical line -146 beats /min

Summary

A 25 yrs old LMP 13/01/21 EDD 20/10/21 G2P1L1 a booked and immunised case with previous first post term elective LSCS for non recurrent indication.

On examination , bilateral pitting pedal edema,a healthy horizontal scar of 10cm noted 3cm above the pubic symphysis,fundal head corresponding to 32 weeks flank full,a single life fetus longitudinally lie cephalic presentation with good fetal heart sound not in labour came for institutional evaluation and management.

Diagnosis

A 25 yrs old G2L1P1A0 with 34 weeks of gestation , with previous LSCS ,having single life fetus longitudinally lie cephalic presentation with good FHS and with no other obstretics and comorbidities cause for institutional management and delivery.

Investigation

Routine investigation: Hb,BG,rh typing ,blood urea sugar,HIV,VDRL ,HBSAG,Usg.

Management : There is no other obstretics and medical comorbidities and also the scar is healthy,so I wanna go for VBAC.

POST DATED DELIVERY

A 18 year old primigravida coming from kalasapakkam w/o Shankar, 26 year old, who is a construction worker belonging to lower socioeconomic status according to MKS. She is a primigravida. Her

LMP-27/12/2020

EDD-3/10/2021

Gestational age-40 weeks and 5 days

Came with chief complaints of amenorrhea with pain abdomen from yesterday morning 4AM

HOPI: She is booked and immunized in nearby PHC.

She complains of pain abdomen from yesterday, originates from the back and radiate to the lower abdomen and then to the thigh, which was gradual in onset and progressive in nature.

No history of bleeding PV

Fetal movements are well perceived by her.

Past history

Not a known case of DM, HTN, Tb, Bronchial Asthama, Epilepsy, Rheumatic heart disease, Thyroid dis: orders.

No history of drug intake other than IFA No history of previous surgeries No history of blood transfusions

Menstrual history:

LMP-27/12/2020

Age of menarche at 13 years, regular cycles of 30 days, flow for 3 days, changes 2 pads per day, not associated with pain or passage of clots

Marital history:

Married since 1 year, conceived within 1 and half months of her marriage, non – consanguineous marriage

Obstetric History:

She is booked and immunized at PHC 1st Trimester:

History of spontaneous conception, pregnancy detected by UPT after 1 and half months of her missed period in PHC.

1st dose of Tt is given

Folic acid tablets were taken regularly

No history of excessive vomiting

No history of fever with rashes

No history of drug intake or radiation exposure

1st trimester scan was done and the pregnancy was confirmed

No history of pain abdomen, bleeding or leaking PV

2nd Trimester:

Quickening felt at 5th month and continued to perceive fetal movements well and taken regular ANC visits

Anomaly scan was done at 6th month and she said it was found to be normal

2nd dose of Tt given 4 weeks apart

Iron Folic acid and calcium were taken regularly; History of easy fatigability

No history of pain abdomen, bleeding or leaking PV

No history of increased volume of micturition, increased thirst, increased appetite

No history of headache, blurring of vision, epigastric pain, vomiting, decreased urine output

No history of dyspnea, palpitations

Underwent routine lab tests and found to be that her Hb was low. So, parenteral administration of iron is given at seventh month on alternate days, 4 doses given

3rd trimester:

Fetal movements are well perceived

IFA and calcium tablets taken regularly

Third trimester scan done and was found to be normal

History of frequent micturition

No history of S/O raised blood pressure, anemia

No history of abdominal pain, bleeding or leaking PV

Underwent routine lab tests and now Hb was improved

Personal History:

Consumes mixed diet, Normal Appetite

Normal sleep pattern, adequate sleep during night and two hours in afternoon

Normal bowel and bladder habits

Family History:

No history of post dated or prolonged pregnancy in the maternal side of her family

No history of DM, HTN, Tb, Epilepsy, Bronchial Asthma, RHD, Thyroid disorders.

No history of twinning in the family

No history of congenital anomalies in the family.

General Physical Examination:

She is conscious, comfortably placed, well oriented, cooperative, moderately built and nourished.

No pallor, No Icterus, Cyanosis, Clubbing, Generalized lymphadenopathy, pedal edema.

Height-145cm

Pre-pregnant weight-48kg

Present weight-54kg

BMI-22.85kg/m²

Breast, Thyroid, Spine normal Vitals:

Temperature-98.6°F

Pulse Rate-90 beats per minute, regular in rhythm, normal volume and character, No R-R, R-F delay, All peripheral pulses are felt bilateral

Blood Pressure-120/70mm Hg, right arm, sitting position

Respiratory Rate-17 cycles per minute Systemic Examination:

Respiratory System:

Normal vesicular breath sounds heard

No altered sounds

Cardiovascular System:

S1, S2 heard

No murmurs

Central Nervous System:

No focal neurological deficit

Abdominal Examination: After getting consent,

She is exposed from xiphisternum to pubic symphysis in a lightened room

INSPECTION:

Abdomen is uniformly distended and flanks are full

Umbilicus is placed centrally and exerted

Linea nigra, Striae gravidarum present

No scars, sinuses, dilated veins

Hernial orifices intact

PALPATION:

No local rise in temperature or tenderness

After correcting dextrorotation, fundal height corresponds to term 32 weeks of gestation

Symphysis fundal height 32cm

Abdominal girth 80cm Leopold's Manoeuvre:

1. Fundal grip-soft, broad mass felt suggestive of breech

2. Umbilical grip

Right side-uniform continuous curved resistance felt suggestive of spine

Left side-Irregular knob like structures-suggestive of limb buds

First pelvic Grip- Hard, not ballotable mass felt-suggestive of fetal head

Second pelvic grip-fingers are diverging-suggestive of that head is engaged.

AUSCULTATION:

FHS are heard, 142 bpm, regular rhythm

SUMMARY:

A 18 year old Primi Gravida, Booked and Immunized came with 9 months of amenorrhea corresponding to 40 weeks plus 5 days of gestation crossing her EDD, regular cycles came with pain abdomen with well perceived fetal movements. She is anemic during 2nd trimester and was treated by giving parental iron. On examination, single live fetus with fundal height corresponding to 32 weeks of gestation flanks full, longitudinal lie, Cephalic presentation and engaged head with good FHS without any medical and Post dated complications not in labour came for Institutional delivery.

Diagnosis:

A 18 year old Primi Gravida with 40 weeks plus 5 days of gestation with post dated pregnancy crossed EDD by 5 days having single live fetus, longitudinal lie, cephalic

presentation, engaged head, with good FHS, without medical or obstetric complications not in labour came for Institutional delivery.

Investigation:

1. Monitor mother

Hb, BP, Pulse, SpO₂, warning signs, symptoms, access
BISHOP score, USG

2. Monitor Baby

CTG, Modified Biophysical Profile, NST

3. Planning for vaginal delivery by induction of laboring

GTMMCH. After routine investigations of Hb concentration, PCV, Blood grouping and typing, HIV, HBsAg, VDRL, urine routine, Blood sugar.

4. Foley's is used for induction.

PGE₂ Gel or Misoprostol- BISHOP < 6

5. If BISHOP score > 6

- Go for ARM only when she is in active labour
- Oxytocin I.v. infusion given.

6. USG-BPD, FC, AC, HC, Wt, Liquor, Placenta

7. DOPPLER-Umbilical abnormal, MCA, Ductus venous.

8. BISHOP

- < 6 - not favourable
- > 6 - favourable

CEPHALO PELVIC DISPROPORTION

Name : Mrs. xyz

Age: 22 yrs

Address: Kilpauk

Occupation: Homemaker

SES: Class 3 [Mod.kupp.scale]

Primigravida

Booked & Immunised at KMC

LMP-07/03/2020

EDD-14/12/2020

Gestational Age -38 weeks +5days

She is primigravida with 9 months of amenorrhea with 38 weeks of gestation for regular AN checkup.

No h/o pain abdomen

No h/o bleeding or draining PV

Fetal movements are perceived well

Obstetric H/O:

H/O present pregnancy:

1st trimester:

- Spontaneous conception after 1 yr of marriage .
- UPT was done at 5 weeks of amenorrhea and the pregnancy was confirmed by ultrasound.
- Dating scan done at 8 weeks .
- NT scan done at 12 weeks and found to be normal .
- H/O morning sickness present .
- Folic acid tablets were taken .
- No H/O fever with rashes.
- No H/O drug intake or radiation exposure.
- No H/O pain abdomen ,bleeding or leaking PV.
- No H/O burning micturition.
- No H/O hyperemesis gravidarum.

2nd trimester:

- Quickening felt at 20 weeks.
- Anomaly scan done at 18 weeks and found to be normal.
- Iron and folic acid ,calcium were taken regularly.
- Immunised with 2 doses of TD 4 weeks apart.

- BP & OGCT were found to be normal.
- No H/O pain abdomen ,bleeding or leaking PV.
- No H/O easy fatiguability ,dyspnea,palpitations.
- No H/O headache,blurring of vision,epigastric pain,decreased urinary output,vomiting
- No H/O increased appetite, increased volume of micturition ,increased thirst.

3rd trimester:

- Fetal movements are well perceived.
- Growth scan was done and found to be normal.
- No H/O pain abdomen, bleeding or leaking PV.
- No H/O S/O PIH,GDM,anemia.

RISK-CORD PROLAPSE

Menstrual history:

- Age at menarche-14yrs
- Regular cycles 28days with 4days of flow,changes 3pads/day,not associated with pain and clots.
- LMP-7/3/2020

Marital H/O:

- Married since 1yr ,non consanguinous marriage.

Past H/O:

- No H/O DM,HTN,thyroiddisorder,bronchial asthma,CVD,epilepsy,rickets,osteomalacia,tuberculosis of hip, Pelvic joints or spine
- No H/O fracture or surgery of pelvis or spine.
- No H/O fracture or surgery of lower limb.
- No H/O tumours of hip or lower limb.
- No H/O of polio in childhood.

Personal H/O:

- Consumes mixed diet & N appetite
- Normal bowel & bladder habits.
- N sleep pattern -8hrs @night ,2hrs in afternoon
- No H/O drug addictions

Family H/O:

- No H/O DM ,HTN,TB in family.
- No H/O bleeding disorders in family.

Summary:

Mrs.xyz,a 22yr old,primigravida with LMP-7.3.2020 &EDD - 14.12.2020 at 38 weeks +5days of gestation who is booked &immunized belongs to class 3 SES(acc to MKS) has come for

regular ANC checkup with 9months of amenorrhea and no other complaints.her 1st &2nd trimester were uneventful .

General physical examination:

- Patient is conscious, oriented ,comfortable,cooperative.
- Moderately built &nourished.
- No pallor ,icterus,cyanosis,clubbing,lymphadenopathy,pedal edema.
 - No deformities of spine
- Breast appears to be normal.
- Thyroid examination:N
- Gait:waddling gait-walking like duck
- She is short stature.
- No shortening of lowerlimb.
- Wt-48kg
- Ht-139 cm
- BMI-24.8 N
- Present wt-58kg wt gain: 10kg

VITALS:

BP-120/78mmHg,Rt.upperlimb,sitting position

PR-86 beats /min,regular in rhythm, N in volume,character,No R-R ,R-F delay ,all peripheral pulses are felt on both sides equally.

RR-18/min,thoracoabdominal type

Temp-98.4°F.

Obstetric examination:

- After getting consent from the patient and she was asked to empty her bladder.
- Abdomen was exposed from xiphisternum to pubic symphysis.
- Examined in dorsal position with semiflexedthigh.

Inspection:

- Abdomen is longitudinally distended, flanks- full.
- Umbilicus in midline &exerted.
- All quadrants move equally with respiration.
- Linea nigra,stria gravidarum seen.
- No scar, sinuses,dilated veins.
- Hernias orifices intact.

Palpation:

- Fundal height corresponds to 32 weeks of gestation which corresponds to term
- Symphysio fundal height-

- Abdominal girth-95cm
- After correcting dextrorotation.

Fundal grip - Broad ,soft,bulky not independently ballotable mass probably breech.

Umbilical grip -

Lt side - smooth ,curved ,hard uniform resistance felt -probably spine .

Rt side -multiple small nodules are felt -probably limb buds .

1st Pelvic grip-hand,round,independently ballotable mass -probably head

2nd Pelvic grip- findings of 1st Pelvic grip confirmed ,hands converging ,not engaged

Abdominal method-head flushed the pubic symphysis.

NO CPD

Mild CPD-flushed

Major CPD -overriding of head with PS

Abdominopelvic assessment of CPD and PV examination to be done.

Auscultation:

- Fetal heart sounds heard at left spinoumbilical line below umbilicus.
- Fetal heart rate 140beats /min.Regular in rate &rhythm.

Systemic examination:

- RS: Normal vehicular breath sounds heard .
No added sounds .
- CVS: S1,S2 heard .no murmurs.
- CNS: No focal neurological deficit.

Summary :

A 23 yr old primi,Mrs. Xyz booked &immunized belongs to class 3 (mks) has come at 32 weeks of gestation with LMP-7.3.2020 and EDD-14. 12.2020 for regular AN checkup with no significant complaints. She was of short stature &her ht is 139cm. On Obstetric examination she has single

live fetus ,longitudinal lie,cephalic presentation ,head is mobile . On abdominal method head flushes the pubic symphysis and Fetal heart rate -140/min.

Diagnosis:

A 23 yr old primi gravida with 32 weeks of gestation with short stature with single live fetus, longitudinal lie, cephalic presentation with good fetal heart sounds with no other medical and obstetrics complications came for institutional evaluation and treatment.

HEART DISEASE IN PREGNANCY

Mrs .XYZ of age 24 yrs from Bangalore, homemaker , completed her education upto class 10 who is a wife of Mr. XYZ , 27 yrs old auto driver belongs to low socioeconomic class is primigravida , booked and immunized at

LMP : 07.07.2020

EDD : 14.04.2021

Came with chief complaints of 2 months of amenorrhea. She has reported for her first ANC visit and is currently asymptomatic who is a known rheumatic heart disease patient.

History of presenting illness

No H/O of breathlessness

No H/O cough with expectoration

No H/ O of haemoptysis

No H/O of fever

No H/O of easy fatiguability

No H/O of swelling of legs

No H/O of chest pain

No H/O of orthopnea

No H/O of PND

No H/ Of syncope

No H/ O diminished urine output

No H/O of any hypertensive dls

Obstetric history

She is a booked and immunized case at PHC .

Pregnancy detected by UPT after ½ month of her missed periods in PHC

H/O spontaneous conception

No H/O excessive vomiting

No H/O fever with rashes

No H/ O drug intake or radiation exposure

No H/O pain abdomen , bleeding , or leaking PV

No H/O chest pain , breathlessness, syncope , fatigue , palpitations, orthopnea,PND , pedal edema

Menstrual history

Age of Menarche- 13 yrs

Regular cycles of 28-30 days

3-4 days of flow

Changes 2-3 pads/ day

Not associated with pain

No passage of clots

Marital history

Married since 1 yr

Non consanguinous marriage

No H/O contraception

Past history

At 12 yrs of age ,she was admitted for fever with severe joint pains . Rheumatic heart disease treated with antibiotics at a local hospital. She has been on 2° prophylaxis of Inj Benzathiane penicillin in every 3 weeks

No H/O DM,HTN,TB,epilepsy ,asthma thyrotoxicosis,recurrent respiratoryinfections,CCF

No H/O surgeries in the past

No H/O blood transfusions

No H/O drug allergy

Personal history

She consumes mixed diet

Normal appetite

Normal sleep pattern

Normal bladder and bowel habits

No H/O of drug addiction

Family history

NO H/O DM,HTN, asthma, TB,heart disease in the family

No H/O of child with congenital malformation

No H/O twining in the family

Summary

A 20yr old primigravida who is booked and immunized with LMP- 7.07.2020 and EDD-14.04.2021 with the 2 months of amenorrhea with 10weeks +three days of gestation spontaneous conception with past history of Acute Rheumatic fever ,currently on monthly Benzathiane penicillin secondary prophylaxis presented for routine ANC visit and is currently asymptomatic

General examination

Patient is conscious, oriented,cooperative,and moderately built and nourished

Mild pallor, noicterus,cynosis,clubbing,generalized lymphadenopathy,pedal edema

JVP not raised

Ht- 155cm

Wt-58kg(prepregnancy)

BMI-24.14Kg/m²

Vitals

Temp_afebrile

PR- 90 beats/min

BP-110/70mmhg

RR_ 16 breaths/min

JVP not raised

Breast,spine,thyroid normal

Gait normal

Systemic examination

CNS- no focal neurological deficit

RS- bilateral equal air entry, vesicular breath sounds in all areas of lung, no added sounds

CVS-S1 loud

Systolic murmur grade 2, abnormal diastolic murmur

Inspection

Trachea appears to be in midline

No chest wall deformity,

no scars, sinuses,dilated vein

Visible apical impulse medial to mid clavicular line

No pericardial bulge

Palpation

Trachea confirmed to be in midline

Apical impulse at 5th intercostal space ½ inch medial to mid clavicular line, tapping in nature

No parasternal heave, thrills palpated

Percussion

Dullness on apex

S1 loud

S2 loud

Auscultation

S1, S2 audible S1 louder than S2, P2 is loud

Mild diastolic murmur in mitral area, grade 2, increased on expiration and decreased on inspiration, presystolic accentuation present.

Opening snap present

No abnormal sounds heard in any other region

Abdominal examination

Inspection

Lower abdomen slightly distended

Umbilicus in midline, inverted

No linea nigra, stria gravidarum present

No scars, sinuses, dilated veins seen

Hernial orifices are intact

Palpation

Soft, non tender

No palpable mass or hepatosplenomegaly

Percussion

No fluid thrill

Auscultation

Normal bowel sounds heard

No bruits heard

Summary

A 24 yr old primigravida who is booked and immunized with LMP 7/7/2020, EDD 14/4/2021 with 2 months of amenorrhea with 10 weeks + three days of gestation spontaneous conception with past history of Acute Rheumatic fever currently on monthly Benzathine penicillin secondary prophylaxis presented for ANC visit and is currently asymptomatic. On examination mild pallor was seen, mild diastolic murmur with open snap and with presystolic accentuation was heard. On s/o mitral stenosis, RHD, NYHA-grade 1

Diagnosis

A 24 yr old primigravida at 10 weeks + three days of gestation presented for ANC, with examination findings mitral stenosis of rheumatic origin in sinus rhythm, with no evidence of infective endocarditis, not in failure NYHA grade 1 complicating the pregnancy with no other medical and Obstetric complications for safe confinement of pregnancy.

Rh -ve PREGNANCY

Mrs. xyz, 29 year old, housemaid by occupation, address,

Educated till 8th std

Blood group: B-ve

Married for 5 years

Husband details

Mr. xyz, 31 year old

Blood group B+ve

Occupation farmer

SES : Low SES

LMP: 28 JULY 2020

EDD: 4 MAY 2021

POG: 10 Weeks, 1 day

G3P1L1A1

She has come with 10 weeks of amenorrhea for her first AN visit and is currently asymptomatic

Past history

No major medical problems like HTN, DM, TB, asthma, thyroid disorders, epilepsy, bleeding problems or drug treatment history

No surgical history

No H/O blood transfusion

Menstrual history

Age of 13 years

Past cycles – regular, 28-30 days -f/b 3-4 days of menses

Not associated with dysmenorrhoea or passage of clots

Marital history

Married for 5 years

Non consanguinous marriage

Obstetric history

G3P1L1A1- 10 Weeks 1 day- POG

H/O first pregnancy

- 2016 (at 25 years of age)
- ANP -unevent full
- Full term normal vaginal delivery
- H/O costly injection administration on 3rd day in the upper arm
- Baby 2.8 kg , cried at birth, live and healthy

Second pregnancy

- 2018(at 27 years of age)
- Spontaneous abortion at 3rd month
- Spontaneous conceptus delivery
- Was managed at village . No H/O prophylactic anti D was given post abortion

History of present pregnancy

Spontaneous conception

First trimester:

Diagnosed by UPT

USG: conformed pregnancy , dating scan done

Routine ANC investigations have been advised , reports awaited

No H/O excessive nausea and vomiting

No H/O fever with rashes

No H/O bleeding PV or discharge PV

No H/O pain abdomen

No H/O burning micturation, increased frequency of micturation

No H/O radiation or teratogenic drug exposure

No easy fatigability

Family history

No H/O diabetes, HTN , asthma, TB, epilepsy, dyscarasia , known hereditary disease in family

No H/O children with congenital malformations

No H/O twinning

Blood group of siblings

Personal history

Consumes mixed diet

Good appetite

Normal sleep patterns

Normal bowel and bladder habits

No H/O alcohol, smoking

No H/O drug allergy

No H/O contraceptive use

General physical examination

The patient is conscious, co operative, well oriented to time , place and person

Patient is moderately built and nourished

Consent was taken and examined in presence of female attendant

Vitals:

Patient is afebrile

PR : 82 beats / minute in right radial artery which is regular in rhythm , good volume , normal in character , no vessel wall thickening , equal on both sides , No

RR, RF delay and all the peripheral pulses were palpable

BP : 120/74 mmHg in right arm

RR : 16 breath /min

JVP : not raised

- Mild pallor
- No icterus , cyanosis , clubbing generalised lymphadenopathy and pedal edema
- BMI : 24.14 Kg / m²
- Gait and spine normal
- thyroid normal
- Breast normal
- No dilated veins over the neck

Other system examination:

RS : normal vesicular breath sounds heard . No added sounds

CVS : S1,S2 heard , no murmurs

CNS : no focal neurological deficit

Abdominal examination (If more than 28 weeks POG)

After getting consent and emptying the bladder , she is exposed from xiphisternum to public symphysis in well lightened room.

Inspection:

Abdomen is longitudinally distended and uniformly

Umbilicus is central and everted , flank are full

Linea nigra present and striae gravidarum present

No scars , sinuses , dilated veins

Palpation

No local rise in temperature or tenderness

After correcting dextrorotation , fundal height corresponds to term 32 weeks of gestation

Symphysiofundal height

Abdominal girth-80 cm

Fundal grip: soft, broad mass felt S/O breech

Umbilical grip:

Right - regular uniform, continuous curved resistance S/O spine

Left- irregular knob like structures S/O limb buds

1 st pelvic grip:

Smooth ,hard , not ballotable , mass felt S/O fetal head

2nd pelvic grip:

Fingers are diverging S/O head engaged

Auscultation

FHS are heard 140 bpm, regular rhythm

Summary

A 29 year old multigravida (G3P1L1A1)

With B+ve at 10 week 1 day POG

Married to B+ve husband, spontaneous conception, presented for routine ANC visit and currently asymptomatic, with no other comorbidities and no significant family history

On examination, a single live fetus with fundal height corresponding to 32 weeks of gestation with flanks full , longitudinal lie , cephalic presentation and head is engaged , came for institute evaluation and treatment

Diagnosis

A 29 year old G3P1L1A1 at 10 weeks 1 day POG with single live fetus , longitudinal lie , cephalic presentation, head engaged , with good LHS without medical and obstetric complications not in labour came for institutional evaluation having Rh-ve isoimmunisation

Investigation:

CBC

Urine – albumin , sugar

Blood – sugar , urea , creatinine , bilirubin.

1. Blood grouping and typing

Reserve blood for delivery from blood bank

If blood grouping is B-ve , evaluate husband's blood group

R.blood sugar

VDRL , HIV , Hbs Ag

2. ICT
3. MCA – PSV DOPPLER
4. USG

ABNORMAL UTERINE BLEEDING

Name: Mrs Sami

Age : 43 years

Address: Erkaumbadu

Occupation: farmer

Husband: Hariharan

Occupation: farmer

S.E.S. – Lower class according to modified Kuppuswamy scale

Obstetric score: P3 L3

Came to opd with chief complaints of heavy menstrual flow for the past 4 months, pain abdomen during menstruation for the past 4 months.

HISTORY OF PRESENTING ILLNESS

The patient was apparently normal before 4 months after which she developed heavy menstrual bleeding during her cyclical menses which was insidious in nature, progressive in nature with

20 days cycle

Each cycle lasting for 7-8 days

She changes 5-6 fully soaked pads per day

She also complaints of waking up at night to change her pads.

Associated with passage of clots.

H/o pain in lower abdomen during menstruation, intermittent in nature, aching type, non-radiating, aggravated on doing work, relieved by taking medication.

H/o fatigue for past 4 months

No h/o any abdominal mass

No h/o vaginal discharge

No h/o intermenstrual, post coital bleeding

No h/o amenorrhea before periods

No h/o any bladder symptoms

No h/o constipation

No h/o significant weight loss

No h/o epistaxis/bleeding gums/bleeding from any other site

No h/o symptoms suggestive of thyroid dysfunction

No h/o intake of OCP

No h/o IUCD insertion

MENSTRUAL H/O

Attained menarche at the age of 14 years.

Past cycle:

30 days cycle

4-5 days of flow

2-3 pads/day

No h/o of pain or passage of clots

Present cycle:

20 days cycle

7-8 days of flow

5-6 pads per day

H/o of pain during menstruation; passage of clots

LMP:: 21-09-2021

MARITAL HISTORY

Married since 26 years

Non consanguinous marriage

OBSTETRIC HISTORY

P3 L3

Conceived after 5 months of mrg;full term vaginal delivery; female 25 yrs

Full term vaginal delivery; female 23 yrs

Full term vaginal delivery; female 20 yrs

CONTRACEPTIVE HISTORY

No h/o any contraceptive use

Permanent sterilization-not done

PAST HISTORY

Not a known case of DM, TB, HTN, epilepsy, asthma, thyroid dysfunction

No h/o any blood transfusion

No h/o previous surgery

No h/o drug intake

FAMILY H/o

No h/o similar complaints in any of the family members

No h/o any bleeding disorders

PERSONAL H/o

Patient consumes mixed diet

Normal appetite

Normal sleep pattern

SUMMARY

A 43 year old, para 3 live 3, came with chief complaints of heavy menstrual bleeding and pain during menstruation for the past 4 months.

GENERAL EXAMINATION

Patient is comfortable, cooperative, moderately built and moderately nourished.

Patient is anaemic

Not icteric

No cyanosis

No clubbing

No lymphadenopathy

No pedal oedema

Breast examination – normal

Spine examination- normal

Thyroid examination- normal

Normal bowel and bladder habits

Height – 150 cm Weight-52 kg BMI -20.3

VITALS

Blood pressure- 120 / 30 mm Hg

Respiratory rate-16 breaths/ min

Pulse rate-80 beats/min

Temperature- afebrile

Other system examination

CVS-S1,S2 normal

RS-NVBS

CNS-NFND

ABDOMINAL EXAMINATION

After emptying the bladder and after getting consent;

INSPECTION

Abdomen- not distended

Umbilicus-midline and inverted

All quadrants move equally with respiration

No scars, sinus or dilated veins/visible pulsations seen over abdomen

PALPATION

Not warm and tender

No organomegaly

Soft

No mass palpable

PERCUSSION

All quadrants resonant

No shifting dullness/ fluid thrill

AUSCULTATION

Bowel sounds heard

Gynaec examination: external genitalia normal , no local lesions

P/V examination- not done

DIAGNOSIS

A 43 year old para 3 live 3 with abnormal uterine bleeding for evaluation with anaemia

MANAGEMENT

Investigations

Haematological test- Hb, PCV, platelets, total count, bleeding time, clotting time, coagulation profile

Thyroid function test

Pap smear

USG- TAS/ TVS

Hysteroscopy guided endometrial biopsy

TREATMENT

General- correct anaemia-hematuric, vitamin and protein supplement

Medical

Non hormonal- NSAIDs, Tranexamic acid, ethamsylate

Hormonal- GnRH/ progestogen/OCP

MIRENA-IUD

SURGERY

Minimal invasive surgery

Ablative procedures

1st generation- TCRE

2nd generation- Radio frequency induced thermal ablation

Hysterectomy

Adenomyosis- adenomyomectomy

Fibroid- myomectomy

Chronic PID- laproscopic lysis of adhesions

Endometriosis- electrocautery or laser vapourisation

Varicose vessels- UAE

Von Willebrand disease- IV desmopressin

FOLLOW UP

Antibiotics

Iron supplements, vitamins, calcium supplements

Remove sutures on 10 th day

Follow up- review with HPE report

Don't strain

Adequate rest

Don't lift any heavy object

Coital abstinence for 6 weeks

Return to hospital if any complications

Look out for any post menopausal symptoms

DIAGNOSIS

43 year old premenopausal women with previous normal vaginal deliveries with AUB with anaemia with no medical comorbidities for constitutional evaluation and treatment.

FIBRIOD

Name: Kannagi

Age: 37 years

Address: Melsozhanur

Occupation: Vegetable vendor

Educational status : middle school

Husband name :segar

Occupation : vegetable vendor

Status: low SES group (class 5)

Obstetrics score :P2L2

Came to OPD with the chief complaints of heavy menstrual flow for the past two months

History of presenting illness

The patient was apparently normal before 2 months after which she developed complaints of prolonged and heavy menstrual bleeding for the past 2 months

Insidious in onset

Intermittent in nature

3 cycles per month

Each cycle lasting for 7 – 8 days

She uses 5-6 per day

Presence of mass protruding through the introitus for the past 2 months

She also complains of pain in the lower abdomen since 2 months during menstruation

Pain

Last for 7-8days

Intermittent

Dull aching type and non radiating type

Aggravated on

Relieved by taking medication

No H/o discharge PV

H/O Post coital pain

No H/o fever, difficulty in micturition

No H/ o retention of urine

No H/o increased frequency of micturition

No H/ o difficulty in passing stools

No H/ o varicose veins

No H/o pedal edema
No H/o Abdominal distension
No H/o loss of appetite or unintentional weight loss
No H/o epistaxis, gum bleeding
No H/o symptoms suggestive of Thyroid dysfunction
No H/o intake of OCP
No H /o IVCD insertion

Menstrual history

Menarche -12 yrs

Past cycles :

30 days cycles
4-5days of flow
2-3 pads per day
History of dysmenorrhoea
No H/o passage of clots

Present cycle :

3 cycles per month
7-8 days of flow
5-6 pads per day
History of dysmenorrhoea
No H/o passage of clots

Obstetrics history

Married life -13 yrs of marriage (married at 24 yrs)

Obstetrics score :P2L2

First child : full term vaginal delivery

Female 12 yrs

Second child :full term vaginal delivery

Female 8 yrs

Contraceptive history

No H/ o any contraceptive used

Past history

Not a known case of DM ,TB ,HTN,Epilepsy ,asthma,Thyroid

No H/o any blood transfusions

No H/o previous surgery

No H/o drug intake

Family History

No H/o similar complaints in any of the family members

No H/o any bleeding disorder

Personal History

Patient Consumes mixed diet

Normal Appetite

Normal sleep pattern

Normal bowel and bladder movements

Summary

A 37 yrs old P2L2 came with the chief complaints heavy menstrual bleeding with mass protruding through the introitus for the past 2 months and pain during menstruation since 2 months

General examination

The patient is conscious, cooperative, well-built and well-nourished.

Mild pallor present

No Icterus,Clubbing, Cyanosis,lymphadenopathy and edema

Weight-55kg

Height-164cm

BMI-20.5

Breast, Thyroid, Spine, skin-Normal

VITALS:

BP-120/80 mm Hg

RR-16 cycles per minute, Thoraco-abdominal type

PR-90 beats per minute

Temperature-Afebrile

ABDOMINAL EXAMINATION:

INSPECTION:

Shape of abdomen-flat distended

Umbilicus-central and inverted

All quadrants move equally with respiration

No scars, sinuses, dilated veins or visible pulsation seen over abdomen

PALPATION:

No local rise of temperature

Tender

Firm, smooth mass, mobile from side to side of 20 weeks of gravida uterus size is felt in the midline.

The upper edge and the two lateral borders are smooth and lower border nor palpable

PERCUSSION:

Dull note is heard over the mass

AUSCULTATION:

Normal bowel sounds are heard

No audible bruit

PER VAGINAL EXAMINATION: Not done

SYSTEMIC EXAMINATION:

CVS:

S1 and S2 sounds heard

No murmurs heard

RS:

Normal vesicular sounds heard

No added sounds

CNS:

No focal neurological deficit

DIAGNOSIS: A 37 Year old lady P2L2 with AUB probably due to fibroid uterus without comorbidities

MANAGEMENT:

INVESTIGATION:

1. Blood Investigation
2. TFT
3. USG
4. Per Speculum, Per vaginal examination (Transmitted Mobility)
5. PAP Smear
6. Endometrial Biopsy
7. UPI to be ruled out

TREATMENT:

Medical:

1. OCP
2. Mifepristone
3. SERM
4. Letrozole

Symptomatic, .

Pain-NSAIDS

Anemia-Ferrous sulphate

Surgical:

1. MYOMECTOMY

- Not completed family
- Nullipara
- Young patients

2. HYSTERECTOMY

- More than 40 years
- Completed family
- Malignancy
- Multipara

UTEROVAGINAL PROLAPSE

Mrs.poongavanam 61 yr old , who is a home maker from TVM with obstetric code:P3 L3 (NUD) belonging to low socioeconomic class came to OPD with chief complaints of mass descending per vaginum for past 5 yrs.

History of presenting illness

The patient was apparently normal before 5 yrs after which she noticed mass descending per vaginum which is gradual in onset , progressive in nature , reducing manually, aggravated by cough ,relieved by lying down.

Prolapse reducible or not

Causes of irreducibility : Edematous , congestion, long standing prolapse

H/O heavy weight lifting

H/O inconvenience while walking

H/O difficulty in initiating act of micturation

No H/ O bleeding PV

No H/O increased frequency of urination

No H/O burning micturation

No H/ O abdominal pain

No H/O vaginal discharge

No H/ O chronic cough, chronic respiratory disease

No H/O pelvic trauma

No H/O CT disorders

No H/O coital difficulty

No H/O fullness in the abdomen

No H/O low back ache

No H/O Sense of inadequate bleeding

No H/O incomplete evacuation of bowel

No H/O postmenopausal bleeding

No H/O constipation

Menstrual history

Attained menopause at age of 50 years

Attain menarche at 15 years

Regular cycle of 5 / 28 days

Used 2-3 clothes per day

No passage of clots

No postmenopausal bleeding

Marital history

Married at age of 21 years

Non consanguinous marriage

No history of use of contraceptives

No H/O sterilization

Obstetric history

First pregnancy

Female child

39 years of age

Normal vaginal term delivery at home

Baby weighed 2.5 kg at birth and was breast fed

No H/O prolonged labour

No H / O instrument used

No H/ O complications during labour

No H/O early resumption of work

Second pregnancy

Female , 36 years of age

Normal vaginal term delivery at home

Baby weighed 2.6 kg at birth and was breast fed

No H/O prolonged labour

No H/O instrument used

No H/O of Complications during labour

No H/O of early resumption of work

Third pregnancy :

Male child, 26 yrs of age

Normal vaginal term delivery at hospital

Baby weighed 2.5 kg at birth and was breast fed

No H/O of prolonged labour

No H/O Of instrumental use

No H/ O of Complications during labour

No H/O of early resumption of work

Past history

She is a known case of diabetes since 8 years and is on oral hypoglycemic drugs

No H/O bronchial asthma, TB , epilepsy, hypertension

No H/O previous surgery

No H/O blood transfusion

Family history

No H/O diabetes , TB ,asthma , epilepsy , CAD, hypertension in the family

No H/O similar complaints in the family

Personal history

She consumes mixed diet

Normal appetite

Normal sleep patterns

Normal bowel habits

H/O difficulty in initiating micturation

Summary

A 61 year old post menopausal multiparous P3L3 who is diabetic on medication with 2 vaginal term delivery , conducted at home and one NV institutional delivery, came with mass descending per vagina , aggravated on coughing and relieved by reducing manually

General examination

Patient is conscious ,oriented ,co operative, moderately built , nourished

No pallor , icterus, cyanosis, clubbing, generalised lymphadenopathy and pedal edema

Weight:48 kg

Height: 150cm

BMI : 21 .3 Kg /m²

Vitals

Temperature: afebrile

PR: 70 beats / min

RR: 18 breaths/ min

BP : 130/80 mm Hg

Spine , breast , thyroid- normal

Abdominal examination

Inspection

Abdomen is not distended

Umbilicus position is central and inverted

Corresponding areas move equally with respiration

No scars , sinuses , dilated veins seen

Hernial orifices are intact

Palpation

No local rise in temperature

No tenderness

Abdomen is soft

No palpable mass

No organomegaly

Percussion

All quadrants are resonant

No fluid thrill

Auscultation

Bowel sounds heard

Pervaginal examination

Inspection

6×8 cm size , pink globular mass protruding outside the introitus (3°)

No ulceration/ keratinization/pigmentation/rugosity /erosion / bleeding over the surface

Appears to bulging on straining

Cough impulse is positive

Palpation

Mass protruding can be reduced manually

No adenexal tenderness

No bleeding on touch

Not getting above the fundus of uterus -3°

Systemic examination

CVS – S1 , S2 sounds heard , no murmur heard

RS – normal vesicular breath sounds heard

No added sounds

CNS – no focal neurological deficit

Provisional diagnosis

A 61 year old post menopausal multiparous P3L3 who is diabetic on medication with 2 vaginal term delivery at home and one NV institutional delivery came with mass per vagina propably UV Prolapse without other medical comorbidities came for institutional evaluation and treatment

Management

Investigation

Routine: Hb , PCV , BT ,CT , platelet count, urine routine, blood sugar, ECG ,HIV HbsAg, VDRL , serum creatinine, RFT, LFT , chest x ray , TFT , blood urea ..

Specific:

- pervaginal examination
- Speculum examination
- PAP smear
- Cervical cytology
- USG

Treatment:

Confirm the diagnosis

Vaginal hysterectomy by removing uterus and cervix due to prolapse

OVARIAN MASS

MRS. Sundari, a 40 years old P3L3 , w/o Povamani who is a housewife and belonging to lower socioeconomic class coming from Tiruvannamalai came to the OPD with the chief complaints of :

1. Lower abdominal mass for past 6 months
2. Pain in the lower abdomen for the past 6 months

3. Menstrual disturbance since last 5 months.

History of presenting illness:

- The patient was apparently normal before 6 months after which she noticed a mass in the lower abdomen. Initially, it was small in size and gradually increased in size till it reaches the current size. The size of the lump does not vary with cough, posture or on straining.
- The mass is associated with pain which is insidious in onset, dull aching type, intermittent in nature, no radiation, aggravated on coughing and whole day heavy work and relieved by taking rest.
- The patient complains of irregular periods since last 5 months. She developed HMB during her cycle menses which was insidious in onset, progressive in nature with throughout the month.
- She changed 5-6 fully soaked pads/day.
- h/o of waking up at night to change her pads associated with clots and associated with pain.
- This episode of HMB was followed by a period of amenorrhea of 3 months duration after which she developed normal menses last month.
- h/o of dyspnea and fatigue
- No h/o urinary symptoms
- No h/o constipation
- No h/o pedal edema, varicose vein
- No h/o dyspareunia
- No h/o vaginal discharge
- No h/o nausea, vomiting, dyspepsia
- No h/o loss of appetite and weight
- No h/o fever
- No h/o yellowish discolouration of skin eyes and urine.
- No H/o lump or discharge from breast
- No H/o bone pain
- No H/o chronic leg pain or swelling
- No H/o significant weight loss.

Past history:-

- No h/o similar complaints in the past
- Not a known case of diabetes, HTN, TB, asthma, CAD, epilepsy and thyroid disorder.
- H/o blood transfusion
- H/o sterilization surgery done 14 years ago
- No h/o any drug intake

Menstrual history:-

- Menarche attained at 13 yrs of age

- Past cycle – regular cycles of 5/28 days , 3 pads/day, not associated with clots and pain.
- Present cycle – her LMP was on 15/09/2020. It lasted for 3 days changed 3 pads/day not associated with clots and pain.

Martial history:-

- Married since 18 yrs non consanguineous marriage.

Obstetrics history:-

P3L3

1st pregnancy – spontaneous conceived after 1 month of marriage, female child ,full term normal vaginal delivery in institution AN and Rn h/o Uneventful .Now the child is 18 Years Old.

2nd Pregnancy: Spontaneous Conception 1Year after 1st Pregnancy .A full term Male Baby by Normal Vaginal delivery in institution. An and pn period uneventfull currently 16 yrs old.

3rd pregnancy: spontaneous conception after 1 yr of previous pregnancy, a full term male bady by normal vaginal delivery in institution. An and Pn period uneventfull currently 14 years old.

No h/o any contraceptive usage in the interpregnancy period.

She underwent sterilization surgery 2 days after the delivery of her 3rd child.

Family history:-

No h/o similar complains in the family.

No h/o any maliganancies in the family.

Personal history:-

Consumes mixed diet

Normal bowel and bladder habits

Normal sleep pattern.

General examination:-

The patient is comfortable , oriented to time, place, and person, cooperative, moderate built and nourished.

The patient is anemic

Not icteric

No cyanosis

No clubbing

No pedal edema

No generalized lymphadenopathy

Ht- 155 cm

Wt-50kg

BMI- 20.8

Breast, thyroid, spine are normal

Vitals:-

Temp-98.4f

RR -16 breaths/min

PR – 80beats/min

BP – 110/70mm Hg

Other systemic examination:-

CVS- s1,s2 heard no murmur

RS – normal vesicular breath sounds heard , no added sound

CNS – no focal neurological defects

Abdominal examination :-

After emptying the bladder and after getting consent

Inspection:-

- Abdomen distended in the lower quadrant
- Umbilicus – midline and inverted
- All quadrants move equally with respiration
- Striatum Albani is seen.

Palpation :-

- Inspectory findings are confirmed
- Not warm not tender
- No organomegaly
- A single mass of size 5*10, occupying hypogastric and both the iliac region , corresponding to 18 wks of gestation, irregular surface, variable in consistency, mobile in hypogastric plane. All the borders are palpated.

Percussion :-

Dullness over the mass

No free fluid present

Auscultation:-

Normal sounds heard

External genitalia – normal

Per speculum examination – not done

Pv examination – not done

Summary :-

A 40yr old P3L3, belonged to lower socioeconomic class came with the chief complains of pain and lump in the lower abdomen since last 6months associated with AUB. No h/o loss of weight and appetite, no pressure symptoms, she was anemic , a mass occupying hypogastric region and both iliac region which is 8*11cm in size , variable in consistency, irregular surface and mobile in horizontal planes. all borders are palpated. On percussion, the mass was found to be dull.

Diagnosis:

A 40 yr old P3L3 came with pain and lump in lower abdomen probably benign ovarian tumor with no other medical complications came for institutional evaluation and treatment.

LEUCORRHEA

Mrs. Kasiyammal, 51 years old P2L1 W/o Ravi who is a housewife, belongs to lower socio-economic class coming from Tiruvannamalai came to the OPD with the chief complaints of

- (i) Excessive white discharge for the past 3 months
- (ii) H/o pain in the lower abdomen and back for past 3 months

HISTORY OF PRESENTING ILLNESS:

The patient was apparently normal before 3 months after which she developed white discharge which was insidious in onset, progressive in nature, mucoid in consistency, no odour, not blood stained, not associated with itching.

H/o of pain the lower abdomen and lower back for past 3 months which was insidious in onset and progressive in nature associated with discharge.

No H/o pruritus

No H/o dysuria

No H/o local drug application

No H/o allergy

No H/o pediculosis

No H/o scabies

No H/o use of intrauterine contraceptive devices

PAST HISTORY:

No H/o similar complaints in the past

Known case of diabetes, hypertension, hypothyroid, renal disease

Not a known case of TB, asthma, epilepsy

No H/o blood transfusion

No H/o drug intake

MENSTRUAL HISTORY:

Menarche attained at 14 years of age

Regular cycles of 5/28 days, 3 pads per day

Not associated with clots and pain

Menopause at the age of 50 years

MARITAL HISTORY:

Married since 30 years

Non-consanguineous marriage

OBSTETRIC HISTORY: P2L1

First child:-

Spontaneous conceived after one month of marriage

Male child, full term vaginal delivery in institution, Antenatal and Postnatal history are uneventful

Child died at the age of 25 years in an accident

Second pregnancy:-

Spontaneous conception 1 year after first pregnancy

A full term male baby delivered by normal vaginal delivery in institution

Antenatal and Postnatal history are uneventful, currently 26 years

CONTRACEPTIVE HISTORY:

She underwent permanent sterilization after the delivery of second child

FAMILY HISTORY:

No H/o similar complaints in the family

No H/o any malignancy in the family

PERSONAL HISTORY:

Consumes mixed diet

Normal bowel and bladder habits

Normal sleep pattern

SUMMARY:

A 51 year old multiparous, postmenopausal women P2L1 with previous 2 NVD who is a known DM, HTN, hypothyroid, renal disorders under medications came with chief complaints of white discharge for the past 3 months and pain in the lower abdomen and back for past 3 months.

GENERAL EXAMINATION:

The patient is comfortable, and cooperative

Moderately built and nourished

The patient is not anaemic

Not icteric

No cyanosis

No clubbing

No pedal oedema

No generalised lymphadenopathy

Height – 155 cm

Weight – 50 kg

BMI – 20

Breast
Thyroid
Spine
Gait



Normal

VITALS:

Temperature – 98.4° F

Respiratory rate – 16 breaths per minute

Pulse rate – 80 beats per minute

Blood pressure – 100/70 mm Hg

OTHER SYSTEM EXAMINATION:

CVS:-

S1, S2 heard

No murmur

RS:-

Normal vesicular breath sounds heard

No added sounds

CNS:-

NFVD

SYSTEMIC EXAMINATION:

Abdominal examination:-

INSPECTION:

Abdomen – not distended, umbilicus is in midline, inverted

All quadrants moves equally with respiration

Flanks free

Healthy midline vertical scar of 4 cm – suprapubic region is present

No sinus, no dilated veins

Hernial orifices – free

PALPATION:

Not warm, not tender

Soft

No organomegaly

PERCUSSION:

No shifting dullness

AUSCULTATION:

Bowel sounds heard

GYNEC Examination:

Pubic hair:- Sparse

Labia majora, minora:- Normal

Urethral meatus:- Clitoris – normal

Vulva:-

No redness

No scratch marks

No signs of inflammation

DIAGNOSIS:

A 51 year old multiparous, postmenopausal women P2L1 who is a known DM, HTN, hypothyroid, renal disorders under medications diagnosed as abnormal vaginal discharge for evaluation.

INVESTIGATIONS:

Routine investigations:-

Blood investigation: TC, DC, ESR, Hb %

Blood sugar

Blood urea

Serum creatinine

LFT

Urine analysis – sugar, albumin

High vaginal swab and cervical smear – Gram stain

SDA, Blood agar

PAP smear

VIA, VIL1, Colposcopy

Specific investigations:-

Wet film

Whiff test

TREATMENT:

Medical treatment:-

Cefoxetine - 2 gm iv every 6 h

Doxycycline - 100 mg iv, followed by oral route

Type of Discharge	Probable Diagnosis	Morphology	Clinical Features	Treatment
Profuse, thin creamy or slightly green in colour, irritating and frothy	Trichomoniasis	Protozoa, actively motile, have anterior flagella central axostyle, moves along the mucous membrane	Discharge causes pruritus, strawberry-vagina (Fig. 2.1). Can have urinary problems, dysuria, dyspareunia	Metronidazole 200 mg t.d.s. $\times$ 7 days. For both partners or 2 g metronidazole single dose
Thick, curdy or flaky discharge	Candidiasis (moniliasis)	Commonest organism— <i>Candida albicans</i> , Gram-positive, grows in acid medium	Vaginal irritation pruritus, vaginal wall congested, discharge visible at the vulva (Fig. 2.2), mucocutaneous junction and in posterior fornix	Anti-fungal agents like imidazole, miconazole, clotrimazole-creams $\times$ 3–6 days; clotrimazole-vag tab 100 mg $\times$ 7 days oral; single dose of fluconazole 150 mg, ideally both partners to be treated
White, milky, non-viscous discharge adherent to vaginal wall, non-itchy, may be foul smelling pH (5–7)	Bacterial vaginosis (termed vaginosis because it is associated with alteration in normal vaginal flora and not due to any specific infection)	Polymicrobial condition caused by <i>Haemophilus vaginalis</i> , <i>Gardnerella</i> , <i>Mobiluncus</i> / <i>Mycoplasma hominis</i> . <i>Mobiluncus</i> is a Gram-positive curved rod with cork-screw spinning anaerobe	Women will have minimal vulval irritation, congestion of vaginal walls (Fig. 2.3)	Metronidazole 500 mg twice daily $\times$ 7 days, ampicillin 500 mg or cephalosporin 500 mg b.i.d. $\times$ 7 days, clindamycin locally or orally 300 mg $\times$ 7 days
Purulent vaginal discharge	Gonococcal vulvovaginitis	Gram-negative intracellular diplococcus- <i>Neisseria gonorrhoea</i>	Discharge, vaginal inflammation, vulval, irritation, pruritus, dysuria, rectal discomfort	Inj. cefoxitin 2 g IM plus probenecid 1 g orally $\times$ 14 days. Followed by doxycycline 100 mg b.i.d or oral tetracycline 250 mg q.i.d. $\times$ 14 days. Treat male partner

INFERTILITY

Mrs .chandra , a 25 year old W/o Mr.Kumar ,30 years coming from thiruvanamali who is house wife by occupation , belonging to upper lower socioeconomic class , studied upto 12 th std .. came to OPD , with C/O inability to conceive for the past two years..

History of presenting illness:

The patient is unable to conceive after 2 years of regular and unprotected intercourse

H/O acne present

H/O weight gain

H/O hair growth around mouth and jaw. (hirsutism) for past 2 years - PCOS

No H/O white discharge PV

No H/O abdominal pain

No H/O fever- PID (tubal block)

No H/O chronic cough or loss of weight – TB – Tubal block

No H/O vulval itching/ genital ulcers-STI – Tubal block

No H/O cold Intolerance, lethargy, hoarseness of voice – hypothyroidism – anovulation

No H/O tremors / palpitations- hyperthyroidism

No H/O abnormal milk secretion – Hyperprolactinemic

No H/O previous abortion, pregnancy, postabortal curettage

No H/O previous abdominal and pelvic surgery- adhesions and tubal block

No H/O drug intake for any psychiatric disorders

Menstrual history:

She attained menarche at 14 years of age

H/O period of amenorrhea followed by profuse bleeding not associated with pain but associated with clots for the past 7 years

Present cycles:

Cycles are irregular

Periods once in two months

Each cycle lasting for four to five days

Changing 4 pads per day(totally 6 cycles per year)

H /O passage of clots not associated with pain

Past cycle :

Regular

28 days cycle

Lasting for 3-4 days

Change 2-3 pads per day

No H/O pain / passage of clots

Marital history:

Married since two years

Non consanguinous marriage

She is living with her husband

Coital history :

Frequency of intercourse : 4- 5 days / week

No H/O dyspareunia

No H/O vaginismus

No H/O contraception use

No H/O hypospadias / epispadias in husband

Aware about fertile period and ovulation

Andrological history:

Husband- Mr. Kumar, 30 years old working as farmer

No H/O heat / radiation exposure

No H/O frequent travel and shift of duties

H/ O smoking (2 cigarettes per day) and occasional alcoholic

No H/ O genital ulcer and dysuria

No H/O mumps in adolescent period

No H/O chronic cough with expectoration

No H/O chest infection/UTI

No H/O trauma to genitals

No H/O diabetes

No H/O drug for hypertension and psychiatric disorders

No H/O surgery in inguinal region ,urethra , hydrocele surgery

No H/O sexual dysfunction

No H/O cloudy urine after ejaculation

Past history:

Not a known case of DM , HTN , CAD ,TB , epilepsy ,asthma

No previous H/O evaluation or treatment for infertility

No H/O surgery/ invasive procedures

Personal history:

She consumes mixed diet

Normal bowel and bladder habits

Normal sleep patterns

Family history:

No relevant family history

Treatment history:

H/O OCP intake for treatment of PCOD (on and off for past seven years)

Summary :

A 25 year old married since 2 years living with her husband came with C /O inability to conceive after 2years of regular and unprotected intercourse with H/O acne , weight gain and hirsutism

General examination:

The patient is conscious, oriented, co operative, moderately built , nourished

No pallor , icterus ,cyanosis , clubbing, generalised lymphadenopathy and pedal edema

Height : 160cm

Weight:70 kg

BMI :27.3

Vitals :

Temperature: afebrile

PR: 85 /min

BP :120/80mm Hg

RR: 16 breaths /min

Examination of

Breast and thyroid- normal

Spine and gait – normal

Other system examination:

- CVS- S1 ,S2 heard , no murmur

RS - NVBS , no added sounds

CNS – no focal neurological deficit

Abdominal examination:

Inspection:

Abdomen not distended

Umbilicus in midline and inverted

All quadrants moves equally with respiration

No scars , sinuses , dilated veins (or visible pulsations)

Palpation:

Soft , not warmth , non tender, no organomegaly

No palpable mass

Percussion:

All quadrants are resonant

No fluid thrill or shifting dullness

Auscultation:

Normal bowel sounds heard

Genital examination:

External genital normal

No focal lesion

Speculum examination: not done

PV , PR : not done

Diagnosis:

A 25 year old married since two years with inability to conceive after 2 years of regular and unprotected intercourse probably a case of primary infertility with PCOD with no medical comorbidities came for institutional evaluation and management.

Investigations:

Hb

Blood grouping / typing

BT/CT

RBS

Blood urea creatinine

Urine – albumin, sugar

TFT , RFT , LFT

Viral markers – HIV , VDRL , Hbs Ag

Insulin levels

Serum testosterone, androgen

Female :

USG abdomen and pelvis

Serum prolactin (>30 ng / ml)

PAP smear

Test for ovulation

Test for ovarian reserve

Test for tubal patency

Male :

Semen analysis

TFT

Serum prolactin

Anti sperm antibodies

Serum testosterone , FSH , LH hormonal assay

USG – scrotum

Karyotyping

Testicular biopsy

Management:

- Counselling the couple about basics of reproduction, fertile period
- Treatment: lifestyle modification+ metformin
- Correct irregular cycles by giving ocps for two to three months
- Ask the patient to come on 2 nd day of cycle and give clomiphene citrate 50 mg / day × 5 days
- Follicular study from day 10 of cycle on alternate days
- Look for increased size of follicles and endometrial thickness
- When follicular size is 18 to 20 mm , give injection HCG 5000 IU
- Ask patient to have intercourse / IUI can be done after 24 hrs of injection HCG
- Await onset of menses or perform UPT , can be done for 6-8 cycles
- If it fails- ART , FSH , HCG therapy
- Laproscopic drilling